
VVUQ Method and Pipeline: A Research Paper

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Abstract.

This paper template holds verification, validation, and uncertainty quantification higher than code generation. It follows the classic IMRAD structure, with one short paragraph per section to be expanded into a full manuscript.

Disclaimer: This work is independent and not endorsed or sponsored by trial sponsors, FDA, CROs, sites, IRBs, regulators, or medical societies; and was generated using Artificial Intelligence.

Keywords

1 Introduction

Embodied clinical code raises an ordering question: should verification precede generation? This paper argues that it must, and presents a method and pipeline that make the ordering enforceable and reproducible.

2 Methods

The pipeline binds each gate to a named external standard, emits ACCEPT, BLOCK, or ESCALATE, and requires the verification fraction to equal 1.0 before an interaction proceeds.

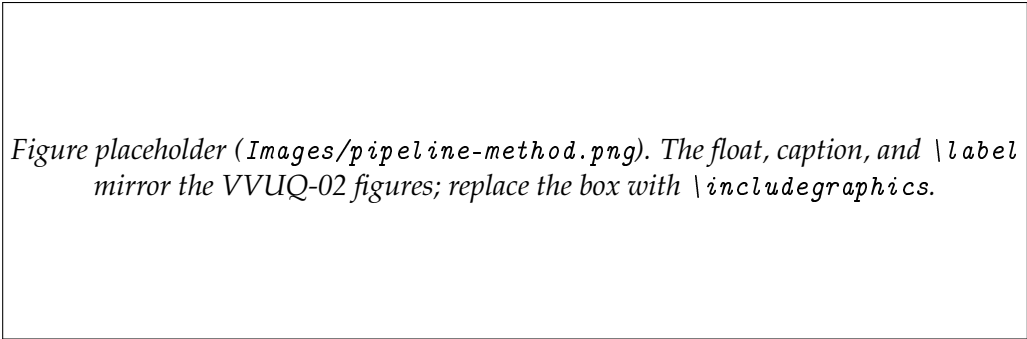


Figure 1: Figure 1. Placeholder schematic of the VVUQ method and pipeline.

3 Results

Work	What it is	Releases	Headline result	DOI
VVUQ-01	method paper plus pipeline	v0.6.0	51/51 tests; fraction 1.0	xxxxxxx
VVUQ-02	ten-gate humanoid codebase	v1.2.0	172/172 tests; mean 93.56	xxxxxxx
VVUQ-03	standalone bill (H. R. 9510)	v1.4.0	verify-before-generate as statute	xxxxxxx

Table 1: Table 1. Headline results across the works (modeled on VVUQ-05 Table 1).

4 Discussion

The results show the gate is feasible at the speed real-time trials demand; the limiting factor is validation-agreement evaluation, not verification, which bounds the method's cost.

5 Conclusions

Verification before generation is both necessary, because embodiment removes the downstream catch, and feasible, because it has been run reproducibly from a fixed seed.

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Ethical Disclosures

The author declares no competing interests. This is an independent draft and is not endorsed, sponsored, or approved by any trial sponsor, CRO, site, IRB, regulator, or medical society, nor by CFR, ICH, or FDA. All supporting numbers are simulation or illustrative results.

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Data Availability

Supporting records are openly available in the `kevinkawchak/cancer-automated` and `kevinkawchak/physical-ai-oncology-trials` GitHub repositories and are archived on Zenodo. All data sets are synthetic or illustrative and reproducible from a deterministic seed; no patient-identifiable information is included.

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